



HEINEKEN Africa Foundation

Application form

Before filling out this application form, make sure you have approval and commitment of the local HEINEKEN operation. Please study the application criteria carefully. This will prevent unnecessary disappointments as the HEINEKEN Africa Foundation can only accept proposals that meet our criteria.

More information can be found on www.africafoundation.heineken.com.

Please answer all questions on this application form in detail (including the requested figures and attachments). Only fully completed application forms can be accepted.

Application criteria

A project proposal can only be submitted to the HAF General Manager if the project:

- is in the field of Mother & Child care or WASH (water, sanitation and hygiene)
- directly improves the health situation for needy communities living in the environment / catchment area of a HEINEKEN Sub Sahara African organization
- cooperates with global and/or local partners with a proven track record (NGOs/GOs/overseas development ministries/international organizations)
- has measurable positive results
- has a sustainable follow-up or clear end goal
- is approved and motivated by the General Manager of the related HEINEKEN operation
- is submitted to the HAF General Manager by the local HEINEKEN account manager
- does not exceed the maximum requested amount of EUR 100,000 per year (with a 3-year maximum)

The HEINEKEN Africa Foundation excludes projects that:

- may lead to a direct commercial benefit for the local HEINEKEN operation
- replace health benefits currently provided to HEINEKEN employees and their family members
- focus on adolescents (in the age of primary and secondary school)
- are research projects, scholarships and medical or health-related events and conferences

Application procedure for 2017

The deadline for the next proposal round is **1 April 2017**.

Proposals received by HAF General Manager	Quality check & revision period	Advisory Board and Trustees meetings	Decision
January – March	April - May	June	First week of July

1. GENERAL INFORMATION

1a. Project name

Orba Cancer Screening and Intervention Center, Enugu

1b. Local related HEINEKEN OpCo

Nigerian Breweries Plc

1c. General Manager OpCo

Name: Nico Vervelde Approval from GM: ☒ Yes

Motivation GM: The project involves establishment of a cancer screening services centre in Orba Primary Health Care Center, Udeni LGA, Enugu state. The project aims to provide access to early detection and prevent death from Cervical and Breast Cancers, Create awareness on different kinds of cancer to empower the target group to adopt and provide cancer treatment and interventions to the indigent and rural populace with detectable signs of imminent cancer.

1d. Name and email of applicant (OpCo account manager)

Name: Dr. John Aigbonohan Email: john.aigbonohan@heineken.com

1e. Name(s) of local or international applicant organisation(s)

Ugo's Touch of Life Foundation (U-TOLF), Enugu, Enugu State

1f. Name and email of project manager from applicant organisation

Name: Mr. Stanley Okere Email: info@u-tolf.org

1g. Short description of project (max. 25 words)

The project involves establishment of a cancer screening services center in Orba Primary Health Care Center, Udeni LGA, Enugu state. It aims to:

1. Provide access to early detection and prevent death from Cervical and Breast Cancers.
2. Create awareness on different kinds of cancer to empower the target group to adopt preventive practices.
3. Provide cancer treatment and interventions to the indigent and rural populace with detectable signs of imminent cancer.
4. Institutionalize sustainable cancer service system within the public healthcare delivery system in Enugu State.

The project will entail:

1. Construction of a 4 room bungalow within Orba Ward Model Primary Health Center
2. Construction of a solar-powered borehole
3. Procurement, installation and maintenance of Phillips Colposcopy machine and Clear Vue 650 Ultrasound machine and accessories including 2yr maintenance contract.
4. Provision of Stabilizers/ UPS
5. Provision of Laboratory testing kits and a consumables (Automated PAP microscope, Spatula, Slides, etc)
6. Provision of treatment advice and other interventions for patient with detectable cancer signs.

1h. Total requested amount from HAF (in EUR)

EUR 99,283.40

1i. Geographical location

Orba town, Udeni LGA, Enugu state.

1j. Estimated time to complete the project (start after signing of contract)

No of months: 18 months add another year to monitor the use, effectiveness and impact of the project
30 months

1k. Theme of this project

☒ Mother & Child Care ☐ Water, Sanitation & Hygiene ☐ Other Health theme

2. THE PROJECT

2a. Problem analysis

Why is this project necessary? Describe the exact problem that you aim to address. Include any available baseline data (data depicting the present situation that will serve as a basis for comparison).

Estimated 500,000 new cases of cancer diagnosis annually in Nigeria. The World Health Organization (WHO) estimates that there are 100,000 new cases of cervical and breast cancer each in Nigeria. Breast cancer presents a typical picture of the enormity of cancer burden on the Nigerian nation (peak age incidence is 42.6 years, mean age of 46.8± 11.5years (of 1094 cases). 12% of patients with Breast cancer are younger than 30. Enugu, with a population of about 3,267,837 as at 2006 census and about 5,000,000 today, there is relatively high number diagnosed during pregnancy and lactation (over 3.1% in Enugu) and relatively high proportion of male breast cancer (1.2% in Enugu). The American cancer society stipulates that women aged 40 years and above should initiate breast cancer screening, at least once a year, in order to detect breast cancer early before any symptom can develop. It is generally accepted that early detection and treatment improves patient outcome. However, even when there is awareness, the poverty level of a larger group of Nigerians makes it near impossible to utilize the screening and treatment services available.

United Nations Report indicates that poverty is still deepening in Nigeria, with over 71.5% of the people earning <1 US dollar a day. In developing countries like Nigeria, clinical services for breast cancer are grossly inadequate and poorly distributed. Only few centers have functioning radiotherapy equipment, and where radiological services are available, utilization are seriously limited by high cost. In Enugu State, there are only three functional mammography centers. Patients also tend to present in the centers very late, when little or no intervention can be done due to lack of awareness, accessibility and other financial constraints. Most of the financial costs are paid through out-of-pocket, which is still the major healthcare financing mechanism in Nigeria. The direct and indirect costs of obtaining care can account for a substantial proportion of total household income for households of people with breast cancer.

U-TOLF has been organising cancer screening programs across Enugu State and has screened over 675 women and treated 82 with minor cancer lesion. Our experience from these outreaches, campaigns and interventions has

demonstrated a huge gap in this aspect of our healthcare delivery system. This appears even more complicated in the midst of growing rates of cancer or its reporting/diagnosis. The problem appears daunting in the rural communities and among the indigent and poor. In one of a more recent medical mission to a community in Enugu State, we recorded 42 positive cervical cancer cases that required emergency surgical interventions out of 121 rural and largely poor women that were randomly screened. Working with some of our partners especially, Joyce and John- a Non-Governmental Organisation in Ebonyi State, we provide surgical support and other treatments to some patients. The U-TOLF HQ conducted its first own-centre surgical intervention for one of the victims on June 21st at its HQ at Enugu. The previous cases from our medical missions and outreaches had been referred to ESUTH at Park lane, Enugu which is located in Enugu town far away from the rural areas where the risk of cancer deaths from late detection is higher due to limited access to screening facilities. With HAF's support this crucial screening services as well as wider coverage of awareness can be achieved at no cost to the people of Enugu State. **How does this intervention relate to MOH policies and priorities? Is the needed equipment according to MOH minimum standards? Will MOH cover the costs of screening beyond the lifetime of the project?....**The needed equipment is actually of an international specification which is in conformity and even exceeds requirements of the MOH policies. See attached MOU with MOH.

What is the population size of Enugu State? As at 2006 census 3,267,837, it's above 5million as at today. It is already embedded in the body of the problem analysis above

2b. Beneficiaries

Who are the beneficiaries of the project? What is the (estimated) size of this group? Include demographic data.

The primary target group are women of sexually active age (about 1,500 women annually) and over across the state . On the average, the estimated number of beneficiaries a month is about 125 women. Also, about 8 medical employees of U-TOLF (4 doctors in training, 2 nurses, 1 pharmacist, 1 medical laboratory scientist) will be deployed to the center once completed. At least 6 civil service health workers of Orba Ward Model Primary Health Center will be engaged to work at the center. All engaged personnel mentioned will enjoy professional training on how to operate, maintain and interpret results of the Clear Vue 650 Ultrasound machine as well as Phillips Colposcopy machine the on use of the equipment for screening by Phillips Medical Services.

Philips will provide training on what? Training of staff on how to operate, maintain and interpret results of the Clear Vue 650 Ultrasound machine as well as Phillips Colposcopy machine the on use of the equipment for screening. Already embedded above.

2c. Aim

Describe the main aim of the project. What will be the contribution of this project in terms of health improvement? E.g. this project will contribute to the reduction of waterborne diseases in community/province/district X. Note: an aim describes what the project will contribute to, not what it exactly achieves! That will be described in 2d.

The project aims to contribute to a reduction in the morbidity and mortality as a result of breast cancer and cervical cancer in Enugu State? YES, because with the screening centre functioning, more cases of pre cancer lesion, will be detected early.

The project aims to contribute to a reduction in morbidity and mortality. While over 80% of cancer deaths could have been detected at late stages in Enugu State between 2010 and 2015, accounting for the high mortality rate of such patients, recent findings from baseline Knowledge Attitude and Practice surveys carried out by U-TOLF and other NGOs in Nigeria, shows that even though the awareness of early cancer screening is increasing in young adult women, most of them are still unaware that breast cancer screening should begin earlier than 40-50yrs and that younger girls can benefit from this as well. Moreover, it has been discovered that about 30% of pre-cancer lesions are missed in about 30% of cases even when they submitted to cancer screening due to inadequacy of the existing equipment to pick the malignant tissue at very early stages. Currently, the techniques used for breast cancer diagnosis such as x-ray mammography, ultrasound and MRIs have shortcomings. The biggest disadvantage with these techniques is the fact that they cannot distinguish between a tumour from healthy tissue or a benign abnormality. This often results in missed tumours and the need to carry out biopsies. This project aims to address this pitfall by partnering with Phillips Medical Equipment Company in Nigeria to employ the use of the new Phillips Clear Vue 650 Ultrasound machine which is known as the “Dream imager for breast cancer”.

The technology at work in this device will shorten the time to accurate diagnosis and will be especially useful among young women where x-ray mammography generally does not perform well. No radiation or contrast media will be used with this device and there will be no pain caused to the women who are examined with it. It does this by combining photoacoustic and ultrasound imaging techniques. Photoacoustic mammography (PAMmography) illuminates the breast with a laser light which is then converted into ultrasound in regions with higher concentration of blood such as the areas around a malignant tumour. The ultrasound produced thus travels from the tumour to the skin where it can be easily detected. In addition, the researchers will develop technology that will generate a three-dimensional image. “The images from the two systems will be combined together. This will result in simultaneous three-dimensional image. Same goes for the Phillips colposcopy machine. See attached images. I DON’T THINK IMAGES ARE ATTACHED

Additionally, this project was conceived to fill the gap of non-existent cancer facilities in Enugu and its environs. It has been noticed that a lot of women in Enugu state are not only ignorant of their cancer status but also do not have access to a health facility where they can be screened and/or treated. Across the 17 local governments of the state and neighbouring states of Enugu, with teeming population of women, it is imperative that this facility be made available to prevent cancer epidemics. Orba Ward Model Primary Health Center in Udeni LGA in Enugu State is centrally located, serves at least 7 other Local government areas and has the highest patient patronage in the area. Hence, we are convinced it will be very apt to site the cancer screening facility there.

Will Philips (Philips Foundation) also contribute with a donation/ financial support? NO , we are only procuring equipment from them

2d. Specific objectives

Formulate the specific objective(s) of the project (max. 3) in terms of change for the beneficiaries. Describe what the project will achieve and be as specific as possible. E.g.: community X (5,000 households) has access to sufficient safe drinking water/ a reduction of 50% of child mortality among children under-five in the catchment area of Health Centre X.

Objective 1 – Establishment of a functional advocacy unit to increase knowledge, attitude and practice of Lifestyle modification for Breast and Cervical Cancers prevention in indigenes of Enugu State and her neighbouring states

Provide some narrative here on how you plan to do this, who will do the awareness, how many people do you target. Where will the sessions take place, etc, U-TOLF has the capacity to carryout awareness during their routine medical outreach round the state, however, a special awareness program per local government area and market towns will be created and implemented to inform the female folks about the screening centre.

Objective 2- Provide and equip Cancer screening center for access to effective facilities for early detection and early intervention to Breast and Cervical Cancers to 1500 women

Provide narrative about the type of equipment (maybe cut and paste from above chapter), how many people will be trained, who will maintain the equipment beyond the lifetime of the project, will staff remain on the MOH payroll etc?

We will use of the new Phillips Clear Vue 650 Ultrasound machine which is known as the “Dream imager for breast cancer” . The technology at work in this device will shorten the time to accurate diagnosis and will be especially useful among young women where x-ray mammography generally does not perform well. No radiation or contrast media will be used with this device and there will no pain caused to the women who are examined with it. It does this by combining photoacoustic and ultrasound imaging techniques Photoacoustic mammography (PAMmography) illuminates the breast with a laser light which is then converted into ultrasound in regions with higher concentration of blood such as the areas around a malignant tumour. The ultrasound produced thus travels from the tumour to the skin where it can be easily detected. In addition, the researchers will develop technology that will generate a three-dimensional image. “The images from the two systems will be combined together. This will result in simultaneous three-dimensional image. Same goes for the Phillips colposcopy machine. A minimum no of 6 personnel will be trained to handle the equipment; their salaries will be paid by the MOH as their staff.

2e. Outcomes

Describe per objective (formulated in 2d) what will be the outcome of that specific objective (max. 3 outcomes per objective). In the table below an example. An outcome is an intermediate step, needed to achieve the objective(s) mentioned in 2d. Give a short description per outcome: how are you planning to undertake the project to achieve the outcome, who will do this, which methodology, who else is involved in getting this done, etc. Also describe why this intermediate step is necessary to achieve the objective.

Objective 1 Community X (5,000 households) has access to clean water

Outcome 1.1	A functioning water source that provides sufficient and clean water to all beneficiaries [Short description]
Outcome 1.2	A functioning water committee that manages the water source [Short description]
Outcome 1.3	The people in the community have improved their knowledge on hygiene [Short description]

Objective 1 Establishment of a functional advocacy unit to increase knowledge, attitude and practice of Lifestyle modification for Breast and Cervical Cancers prevention in indigenes of Enugu State and her neighbouring states

Outcome 1.1	Establishment of a functional advocacy unit- The partnership between Enugu State Government, HAF and U-TOLF (an NGO) to implement advocacy to people across Enugu State & development of the Screening center will ensure a functional coordinating committee is set up to drive the project and will also guarantee the commitment of all stakeholders which will result in release of adequate resources for advocacy campaigns. If you already have an MOU to support this please attach. The MOU is available, see as attached Resources will include Flyers, Public Address systems, radio jingles etc. This will be done in
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	<i>hospitals, markets, town hall meetings and residential neighbourhoods. This will guarantee sustainability of the project.</i>
Outcome 1.2	Establishment of functional referral centers anchored by the Enugu State Health Management Board to ensure early referral from hospitals to proposed Cancer Screening Center- <i>The partnership in outcome 1.1 above will ensure that a closed system of referral between primary care centers and private health centers are quickly effected to foster early detection and intervention through screenings.</i>
Outcome 1.3	Improved knowledge and health seeking behaviour about preventing Cervical and Breast Cancer. <i>This will improve knowledge of the people and stimulate significant behavioural change among indigenes. What sort of health seeking behaviour are you promoting? What is the size of your target group for this outcome? Safe sex and routine screening for about 1500 women each year</i>

Objective 2 Provide and equip Cancer screening center for access to effective facilities for early detection and early intervention to Breast and Cervical Cancers to 1500 women

Outcome 2.1	Construction of Cancer Screening building in Orba Primary Health Center as a base for Breast and Cervical Cancer screening- This will entail the construction of a 4 bed room building with an existing Primary health care center comprising of laboratory section and radiological section. This will ensure continuity of the services since it will be owned and run by the Enugu State Government through the Health Management Board. The UTOLF NGO will only provide technical support and bring their experience with advocacy and focused improved radiologic screenings on board.
Outcome 2.2	Procurement and installation of medical equipment for screening and testing. <i>This will include laboratory equipment such as Microscopes and Slides as well as radiologic equipment such as the new Phillips Clear Vue 650 Ultrasound machine which is known as the "Dream imager for breast cancer". All these will ensure enhanced chances of early detection pf previously missed breast or Cervical lesions and eliminate incidence of late detection. Has there been a comparative assessment which has concluded that it should be the Philips machine, if so please attach report, Based on industry standard and rating, Philip is rated high in medical equipment manufacturers.</i>
Outcome 2.3	Improve access to Breast & Cancer Screening and early detection- <i>The outcome 2.1 and 2.2 above will lead to improved provision of access to better screening facilities at affordable cost. Majority of the women and children are not only ignorant of their cancer status but also do not have access to a health facility where they can be screened and/or treated. Across the 17 local governments of the state and neighbouring states of Enugu, with teeming population of women, it is imperative that this facility be made available to prevent cancer epidemics. Orba Ward Model Primary Health Center in Udeni LGA in Enugu State is centrally located, serves at least 7 other Local government areas and has the highest patient patronage in the area.</i>

2f. Activities

Describe the exact activities that are necessary to achieve the outcomes formulated in 2e. Include the timing. See below the example of the activities necessary for outcome 1.1. Note that it is best to write activities down in an active form, starting with a verb.

Make a separate set of activities for project monitoring. In general, HAF would like to see at least one year of impact monitoring following the installation of a water source/ investment in a clinic/ training for health staff, etc. Also plan for an end evaluation. Note that projects that request EUR 100,000 or more need to include an evaluation done by an external party.

Outcome 1.1 A functioning water source that provides sufficient and clean water to all beneficiaries

#	Description of activity	Timing
1.1.1	Select the location of water source and conduct a geophysical survey	Month 1
1.1.2	Select supplier	Month 1
1.1.3	Purchase the necessary equipment	Month 2-3
1.1.4	Install and test the water source	Month 4-6
1.1.5	Sign MOU on maintenance with X	Month 1-2
1.1.6	Handover the water source to the community and the water committee	Month 7

Please fill in a table with all the activities for each outcome (as defined in 2e).

Outcome 1.1 Establishment of a functional advocacy unit-

#	Description of activity	Timing
1	Select Location to situate the Cancer screening center as base for advocacy	August 2016
2	Select & engage technical Partner & Enugu State government & gather resources	October 2016
3	Sign MOU with partners	July 2017
4	Engage health care providers in Enugu State and commence Advocacy	August 2017

Outcome 1.2 Establishment of functional referral centers anchored by the Enugu State Health Management Board to ensure early referral from hospitals to proposed Cancer Screening Center-

#	Description of activity	Timing
1	Select & engage x health care providers	July 2017
2	Forum with health care providers to establish functional referral system	August 2017
3	Establish reporting system	August 2017

Outcome 1.3 Improved knowledge and health seeking behaviour about preventing Cervical and Breast Cancer.

#	Description of activity	Timing
1	Establish a periodic sentinel survey schedule	July 2017
2	Collate and interpreted data from survey	August 2017
3	Evaluate data and report for intervention	Quarterly

Outcome 2.1 Construction of Cancer Screening building in Orba Primary Health Center as a base for Breast and Cervical Cancer screening-

#	Description of activity	Timing
1	Select a location for construction of center	August 2016
2	Select a construction company	January 2017
3	Construct borehole for water source <i>Has a geophysical study been done, is it known there is water? If so please attach survey report. It is now attached</i>	July 2017
4	Construct building for cancer screening	October 2017

Outcome 2.2 Procurement and installation of medical equipment for screening and testing.

#	Description of activity	Timing
1	Select a supplier for equipment and furniture	August 2016
2	Purchase, supply and install & test equipment and furniture <i>Has this already been done? NO. This is an action to be carried out if project is approved and funds are provided HAF</i>	January 2017
3	Construct borehole for water source	July 2017
4	Construct building for cancer screening	October 2017

Outcome 2. Improve access to Breast & Cancer Screening and early detection-

#	Description of activity	Timing
1	Conduct hospital, home & markets awareness to create awareness about new screening center	August 2016
2	Organise assisted transportation to indigenes to the new center.	Continuous
3	Provide reward for patients who refer neighbours and families	Continuous
4	Provide sample transfer facilities for screenings conducted outside Udeno local government where the center will be situated. This is unclear, please explain The facilities include examination couches, screens, swabs and other movable screening materials and consumables for cervical cancer screening outreaches	Continuous

Project monitoring and end evaluation

#	Description of activity	Timing
	Fully operational cancer screening centre	March 2017
	No of women screened (an estimated 1,500 women screened). Yes	December 31, 2018
	No of women treated (an estimated 263 women treated) yes	December 31, 2018
	Reduction in mortality from cancer in Enugu and its environs	December 31, 2018

3. END OF PROJECT AND SUSTAINABILITY

3a. Sustainability

Define the end of the project. How will this project continue after the HAF funding stops?

The project will end upon commissioning of the equipped Orba Cancer Screening Center and maintenance for 2yrs post-construction and installation of equipment when a handing over will be done to the Enugu State Government by HAF. At the end of the project, the facility will be sustained and operations will be continued under the supervision of the State Health Service Board for the Enugu State Government and screening services will be provided free of charge. However, the Enugu State government will continue to provide staff salaries and maintain the facility. UTOLF will continue to provide services free of charge at the center. There is already a partnership existing between Enugu State Government and UTOLF which suffices to support with staff salaries and logistics for advocacy.

3b. Maintenance plan

How will this project (including the funded hardware) be maintained and by whom? Include a maintenance plan

A maintenance contract will be drawn with the supplier of equipment while the Enugu State government will be responsible for maintenance of other infrastructure. The building will be cited within an existing Primary Health Care Center owned and run by the Enugu State Government to ensure sustainability.

3c. Evaluation

To conclude and finalize the project an end evaluation needs to be done. When do you plan this end evaluation and what will be the method(s) used. Note: projects requesting over EUR 100,000 need to include an evaluation done by an external party.

Timing (month + year) December 2018

Method Sentinel Survey, HAF tracker

4. PARTNERS AND GOVERNANCE

4a. Partner and stakeholders

What is the (local or international) applicant organization in this project? What is the reason to work with them? If possible, please attach references and the website of the applicant organization. Are there any other external stakeholders involved in this project?

U-TOLF: provide background information on the organisation, role in this project, website etc www.u-tolf.org

Philips medical Services: Technical partners for sales, training and maintenance

MOH/ Enugu State Government: Responsible for the wage bill of personnel

Joyce John Cancer Foundation: clinical support consultant, www.uzagwu.org

4b. Governance

How will the project be managed, monitored, evaluated? Who will take up which responsibility?

Role	Organization	Name	Responsibility
Project manager	U-TOLF, Enugu	Mr. Stanley Okere	Administrative Management
Account manager OpCo	HAF	Dr. John Aigbonohan	Quarterly tracking & reporting
Supply, Installation and staff Training on Equipment	Phillips Medical Services	Dr. Osayomore Ossuetta	Training & Installation
Clinical Staff coaching	Joyce John Cancer Foundation	Dr. Uzo Agwu	Clinical Management
Project manager	U-TOLF, Enugu	Mr. Stanley Okere	Administrative Management

5. BUDGET AND FINANCING SCHEME

5a. Currency

The Foundation will only provide contribution in euro's or US dollars to minimize exchange rate losses.

Choose your preferred currency ☒ **EURO** ☐ **US DOLLAR**

5b. Budget

Amount (in preferred currency)

Total costs of project

Include a specified budget as attachment. : make sure your budget includes the needed staff costs, office running costs, etc. Also ensure that the budget is fully aligned with the activities listed in section 2f to avoid gaps in funding later on.

5c. Financing scheme

Include the cash and also the in kind contributions.

Total amount required for this project	114,918.4
Contribution applicant organization	15,635
Contribution OpCo	Click here
Contribution (local) government	
Contribution(s) by other funds/donors (incl. names and amounts)	Click here
Requested amount from HAF	98,283

Instalments

Include the preferred instalments of the budget with the estimated timing in the table below. Note: HAF maintains the right to delay instalments in case implementation is delayed or halted. This will be stipulated in the contract.

# of instalment (max 4)	Amount	Estimated timing (month + year)
I. Construction of building & Part Procurement/ installation of equipment	63,684.05	July 2017
II Part procurement of Equipment & Maintenance	35,599.35	October 2017
II		
IV		

6. REMARKS

Fill in any other information that adds value to this proposal.

While over 80% of cancer deaths could were detected at late stages in Enugu State between 2010 and 2015, accounting for the high mortality rate of such patients, recent findings from baseline Knowledge Attitude and Practice surveys carried out by U-TOLF and other NGOs in Nigeria, shows that even though the awareness of early cancer screening is increasing in young adult women, most of parents are still unaware that breast cancer screening should begin earlier than 40 -50yrs and that younger girls can benefit from this as well. More so, there it has been discovered that about the pre-cancer lesions are missed in about 30% of cases even when they submitted to cancer screening due to inadequacy of the existing equipment to pick the malignant tissue at very early stages. Currently, the techniques used for breast cancer diagnosis such as x-ray mammography, ultrasound and MRIs have short-comings. The biggest disadvantage with these techniques is the fact that they cannot distinguish between a tumor from healthy tissue or a benign abnormality. This often results in missed tumours and the need to carry out biopsies. This project aims to address this pitfalls by partnering with Phillips Medical Equipment Company in Nigeria to employ the use of the new Phillips Clear Vue 650 Ultrasound machine which is known as the "Dream imager for breast cancer" .

The technology at work in this device will shorten the time to accurate diagnosis and will be especially useful among young women where x-ray mammography generally does not perform well. No radiation or contract media will be used with this device and there will no pain caused to the women who are examined with it. It does this by combining photoacoustic and ultrasound imaging techniques. Photoacoustic mammography (PAMmography) illuminates the breast with a laser light which is then converted into ultrasound in regions with higher concentration of blood such as the areas around a malignant tumour. The ultrasound produced thus travels from the tumour to the skin where it can be easily detected. In addition, the researchers will develop technology that will generate a three-dimensional image. "The images from the two systems will be combined together. This will result in simultaneous three-dimensional image. Same goes for the Phillips colposcopy machine.

7. ATTACHMENTS

Send the following attachments together with the completed application form:

- *Required: Specified budget*
- *Required: reference letters on applicant organization and registration certificate/ document*
- *Optional: MOU with collaborating (governmental) partners*
- *Optional: Photo(s) of the preparation of the project/project location*
- *Optional: Additional information of the applicant organization (articles of association, most recent audit report, annual report, publications etc.)*
- *Optional: Preparatory reports, if these have already been undertaken: baseline survey, needs assessment, geophysical study, etc.*

Please make a list of actually attached documents

--Budget

-Pictures of site for construction of building center

...Picture of proposed Phillips Clear Vue 650 Ultrasound machine

--Picture of Phillips Colposcopy machine

--MOU with MOH

--MOU with Enugu State Teaching Hospital

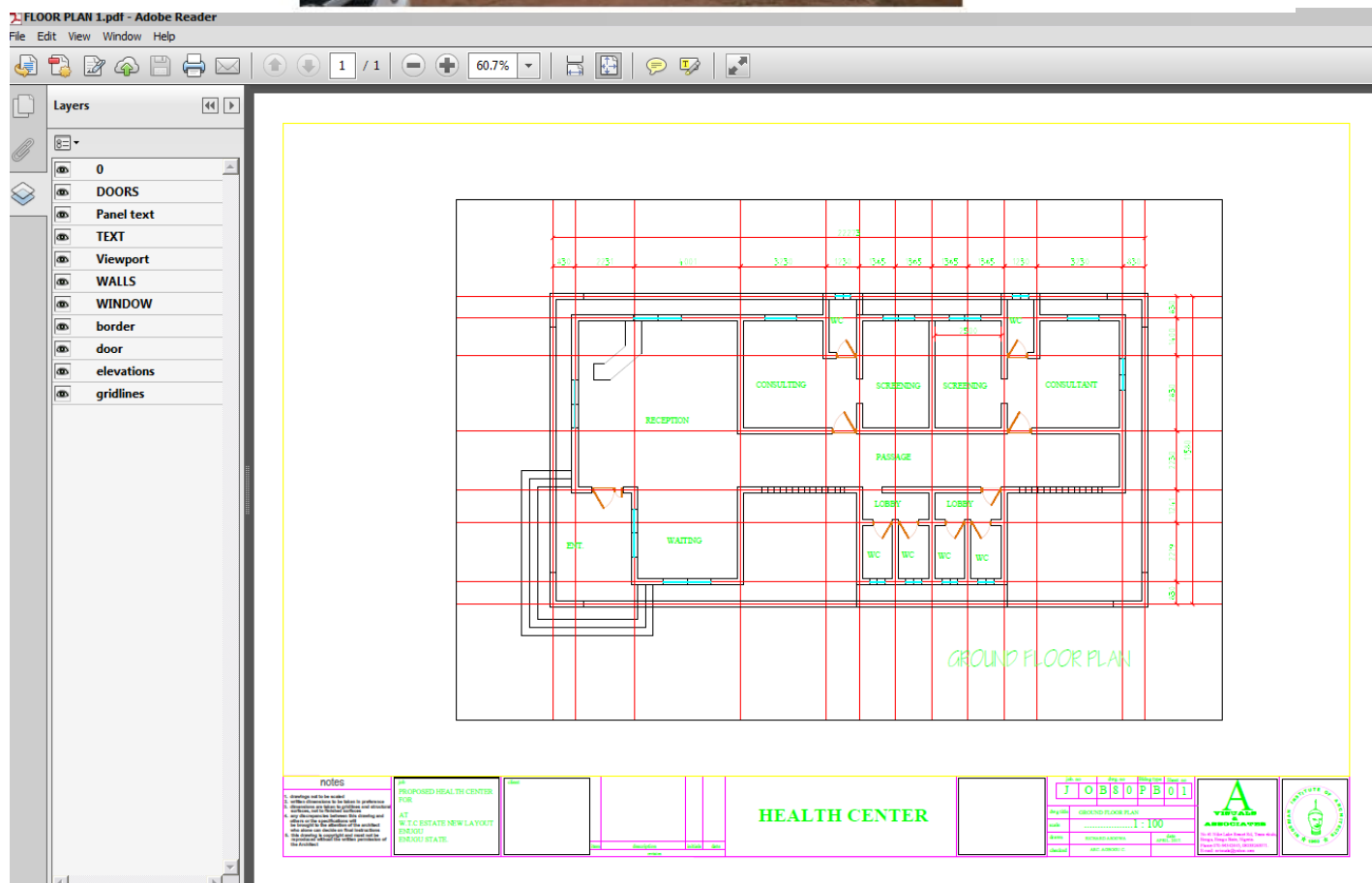
-Floor plan and elevation for proposed building

-Geophysical survey for borehole

--Comparative study

...Certificate of incorporation UTOLF

Only the OpCo Account Manager can submit the final proposal including the attachments to the HAF General Manager.



Philips ClearVue 650 Ultrasound Machine

The Philips ClearVue 650 is a midrange 4D OB/GYN and shared service ultrasound machine. It is powered by Active Array technology, is easy to use, and has best in class automation features. The Philips ClearVue 650 replaces the **HD9** and is visually identical to the **ClearVue 550** except it has a larger monitor.



Philips ClearVue 650 Price
Quote

Lease / Rent the Philips
ClearVue 650

Specs	Clinical Images	Transducers	Features	Imaging & Applications	Video
System Overview					
Year Launched	2013				
Estimated Market Price (\$)	Mid				
Monitor (Inch)	19" LCD				
Tilt/Rotate Adjustable Monitor	Yes				
Monitor Resolution	1280*1024				
Image Size Resolution					
Touch Screen (Inch)	No				
Trackball or Trackpad	Trackball				
CP Back-Lighting	Yes				
Weight	115lbs(52kg)				
Probe Ports	4				
Battery	No				
Boot-Up Time					
Sleep Mode (Quick Start)	No				
Maximum Depth of Field	30cm				
Minimum Depth of Field					
Cart (HCU)	No				
Independent Steer & Lockable Wheels	Yes				
Connectivity					
DICOM 3.0	Yes				
DICOM SR_Cardiac	Yes				
DICOM SR_Vascular	Yes				
DICOM SR_OB/GYN	Yes				
JPEG, WMV, & AVI	Yes				
USB	Yes(3)				
HDD/SDD	320GB				
DVD/CD RW	Yes				
Wireless LAN	No				



Philips Goldway SLC-2000B & WS-2000 Digital Video Colposcope Imaging System

Affordable Colposcopy Examinations Help Prevent Cervical Cancer

Download Philips Goldway SLC-2000B & WS-2000 Digital Video Colposcope Imaging System Brochure

Category: Colposcope.

Tag: Philips Goldway.

ORBA CANCER SCREENING CENTER PROJECT ENUGU

TO

BE IMPLEMENTED IN PARTNERSHIP BETWEEN HAF AND ENUGU STATE GOVERNMENT AND UTOLF
COST BREAKDOWN FOR THE ORBA CANCER SCREENING CENTRE

DESCRIPTION	QUANTITY	COST (NAIRA)	COST(EURO)
CONSTRUCTION			
CONSTRUCTION OF 3 BEDROOM BUNGALOW <i>Can you provide a budget breakdown here as this is a large sum of money, YES With detailed drawings</i>	1	17.000.000,00	52.147,24
EQUIPMENT			-
SLC-2000B VIDEO COLPOSCOPE	1	2.459.105,00	7.543,27
TROLLEY	2	303.387,00	930,63
DESKTOP & SOFTWARES	2	382.398,00	1.173,00
SPECIALIZED COLOUR PRINTER	1	158.224,00	485,35
CLEARVUE 850 ULTRASOUND SYSTEM	1	11.605.274,00	35.599,00

STABILIZER/UPS	1	458.000,00	1.404,91
TOTAL		32.366.388,00	99.283,40
TOTAL AMOUNT REQUESTED FROM HAF		32.366.388,00	99.283,40
CONTRIBUTION FROM UTOLF & ENUGU STATE GOVERNEMENT			
Generator		1.885.910,00	5785
Office furniture		1.946.220,00	5970
Laboratory equipment		1.264.880,00	3880
Staff Salaries			Continuous
TOTAL AMOUNT CONTRIBUTION BY UTOLF		5.097.010,00	15635
TOTAL AMOUNT NEEDED FOR THE PROJECT		37.463.398,00	114.918,40

CAC/IT/NO 80343



CORPORATE AFFAIRS COMMISSION
FEDERAL REPUBLIC OF NIGERIA

Certificate of Incorporation

of the Incorporated Trustees of

UGO'S TOUCH OF LIFE FOUNDATION

I hereby certify that

MRS. MONICA UGWUANYI, MRS. PRISCILLA NKEMKAH, MRS. EVA-MARIA CHUKWU

the duly appointed trustees of UGO'S TOUCH OF LIFE FOUNDATION have this day been registered as a corporate body, subject to the below mentioned conditions and directions.

Given under my hand and the Common Seal of the Corporate Affairs Commission at Abuja this 26th day of August, 2015

CONDITIONS AND DIRECTIONS

This certificate is liable to cancellation should the objects or the rules of the body be changed without the previous consent in writing of the Registrar General or should the body at any time permit or condone any divergence from or breach of such objects and rules.

Note:

This certificate does not bestow upon the Organization the right to establish any institution, engage in any business and the like without permission from the appropriate authority



BELLO MAHMUD

**MEMORANDUM OF UNDERSTANDING
(MOU)**

ON

**ESTABLISHMENT OF CANCER SCREENING
CENTRES**

&

DEPLOYMENT OF HEALTH PERSONNEL

BETWEEN

**UGO'S TOUCH OF LIFE FOUNDATION
(U-TOLF)**

AND

ENUGU STATE MINISTRY OF HEALTH

MEMORANDUM OF UNDERSTANDING

This memorandum of understanding is made the 24-day of April-2017

BETWEEN

UGO'S TOUCH OF LIFE FOUNDATION (U-TOLF), a Non-Governmental Organisation incorporated under the Laws of the Federation of Nigeria with registered office address at Old Governor's Lodge, Off Abakaliki Road, Behind PPSMB, GRA, Enugu (hereinafter referred to as "The Foundation") of one part

AND

ENUGU STATE MINISTRY OF HEALTH, a government establishment with the statutory power of programme/policy initiation and implementation on specific areas of human life on behalf of government; located at the State Secretariat, PMB 1034, Enugu (hereinafter referred to as "**The Ministry**") of the other part

WHEREAS

1. The Foundation has expressed its interest in bringing in, from time to time, some companies that would partner with her to introduce/establish health institutions in the state, either in the rural or urban area.
2. The Ministry is desirous of supporting the Foundation through mobilizing its health personnel to such new health institutions, whether in the rural or urban area.
3. The Foundation is willing and ready to call upon the Ministry any time a new health institution is introduced/established.

4. The Ministry has accepted to be responsible for the salaries of its health personnel mobilized to such new health institutions.

**NOW THE PARTIES TO THIS MEMORANDUM OF UNDERSTANDING
HAVE AGREED AS FOLLOWS:**

1. That Mr Stanley Okere, Consultant to U-TOLF shall, for the purposes of this Memorandum of Understanding, act as the Director/Representative of the Foundation.
2. That the Foundation shall herein, prior to and at the execution of this agreement, be accorded unfettered access to the Commissioner for Health, if need be.
3. That at the execution of this Memorandum of Understanding, the Foundation shall always receive prompt response whenever it calls upon the ministry.
4. That the Ministry shall remain responsible for the salary payment of any or all of its health personnel deployed to any or all such institutions newly introduced/established by the Foundation.
5. That the agreement becomes effective upon the date of the completion of the screening centre and shall remain valid during the centres' life span.

IN WITNESS WHEREOF UGO'S TOUCH OF LIFE FOUNDATION (U-TOLF)
has hereunto set her hand and seal the day and year first above written.

THE COMMON SEAL of the within named "**UGO'S TOUCH OF LIFE
FOUNDATION, ENUGU**" was duly affixed, sealed and delivered

IN THE PRESENCE OF:

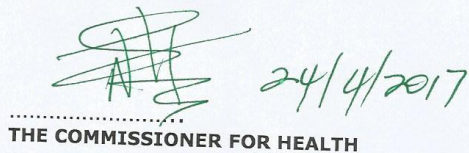

.....
DIRECTOR

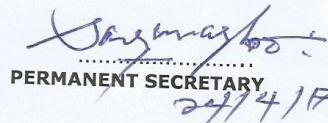

.....
DIRECTOR/SECRETARY

&

THE COMMON SEAL of the within named
"MINISTRY OF HEALTH, ENUGU" was duly affixed, Sealed and the
delivered

IN THE PRESENCE OF:


.....
THE COMMISSIONER FOR HEALTH


.....
PERMANENT SECRETARY

MEMORANDUM OF UNDERSTANDING

BETWEEN

**UGO'S TOUCH OF LIFE FOUNDATION, ENUGU
(UTOLF)**

AND

**ENUGU STATE UNIVERSITY OF SCIENCE AND
TECHNOLOGY TEACHING HOSPITAL PARKLANE
(ESUTTH)**

This **MEMORANDUM OF UNDERSTANDING** is for the purpose of providing treatment of Minor Lesion of patients screened and referred by UTOLF to ESUTTH On ~~1.5.11~~^{15th} day of ~~March~~^{April}, 2016

Between

UGO'S TOUCH OF LIFE FOUNDATION

(hereinafter referred to as "UTOLF")

And

ENUGU STATE UNIVERSITY OF SCIENCE AND TECHNOLOGY TEACHING HOSPITAL

(hereinafter referred to as "ESUTTH")

UTOLF AND ESUTTH are hereinafter collectively referred to as "the parties" and individually also referred to as "the party"

WHEREAS

UTOLF is a philanthropic non-governmental organization that has been partnering with the governmental and non-governmental organizations to provide social and health services through medicals outreaches, visitations and provision of essential resources for the succor of the underserved and vulnerable communities in Enugu state.

ESUTTH is a State owned government hospital which renders improved tertiary health services to the masses, and also serves as a teaching hospital to resident doctors, medical and nursing students. It also serves as a health institution with the capacity of attending to referrals from hospitals from far and near.

The parties are desirous of carrying out this partnership in collaboration with other relevant Civil Society Organizations (CSOs) in the health sector of Enugu State, with having defined roles and responsibilities in actualizing these set understandings.

IT IS HEREBY AGREED AS FOLLOWS:

OBJECTIVES

The objectives of exercise include, but are not limited to, the following:

1. To treat any detected, or referred cases of Minor Lesion from U-TOLF after free screening have been conducted.
2. To Promote partnership among the parties in the overall interest of the clients seeking health services.

3. To admit and treat at the instance of referral, cases of minor lesion and bills afterwards forwarded to U-TOLF to defray.

OBLIGATIONS OF THE PARTIES

Now, THEREFORE, the parties hereto agree as follows:

➤ Obligations of UTOLF

UTOLF shall make all best efforts to screen patients either during medical missions or emergency intervention, and send referrals to ESUTTH.

- a. Promotion of partnership among the parties in the overall interest of the clients seeking health services.

➤ Obligations of ESUTTH shall:

- a. make all best efforts to implement the following during the period of the memorandum through:
Treatment of any detected, or referred cases of Minor Lesion from U-TOLF after free screening have been conducted.
- b. Promotion of partnership among the parties in the overall interest of the clients seeking health services.
- c. Admission and treatment at the instance of referring cases of minor lesion and afterwards forward bills to U-TOLF to defray.

GENERAL TERMS

Confidentiality

Neither of the parties is to disclose, directly or indirectly, any confidential information received from the other party to any third party without written consent of the party that disclosed such information. This clause should not apply to information which is: in the public domain; in the possession of the receiving party before such divulgence has taken place; obtained from a third party who is free to divulge the same; and or required to be disclosed by the operation of law.

Dispute Resolution

Any dispute concerning the subject matter of this document is to be settled by full and frank discussion and negotiation between parties.

Disclaimer

This memorandum is not intended to and does not create any contractual rights between these parties. However, the parties in good faith pledge their mutual best efforts to achieve the goals and objectives set forth above.

Amendment

This memorandum of understanding is at-will and may be amended or modified at any time with the mutual consent of authorized representatives of the parties.

Termination

Either party may not terminate this memorandum at any time without assigning any reason whatsoever by written notice to other party of its desire to terminate this memorandum.

Miscellaneous

The parties to strictly comply with all decrees, laws and regulations of the government of Nigeria pertaining to the activities undertaken through this memorandum.

This Agreement shall supersede all prior agreements, understanding, arrangements, promises, representations, of any form, or nature whatsoever, whether oral or written and whether explicit or implicit, which may have been entered into by both parties prior to the execution of this agreement. This memorandum of understanding shall become effective on the date signed by both parties and shall be referred to as its effective date.

IN WITNESS WHEREOF the parties hereunto have signed hereunder the day and year first above written.

Signed by the authorized representative of **UGO'S TOUCH OF LIFE FOUNDATION, ENUGU**

Stanley Okoro
Name

[Signature]
Director

Barr. (Mrs) Onyinyechi Anichukwu
Name

[Signature]
Secretary

Signed by the authorized representative of **ENUGU STATE UNIVERSITY OF SCIENCE AND TECHNOLOGY TEACHING HOSPITAL, ENUGU**

GABRIEL NJEZE
Name

[Signature]
Director

CHRIS J. NGENE ESQ.
Name

[Signature]
Secretary

GEOPHYSICAL AND HYDROGEOLOGICAL
INVESTIGATION FOR GROUNDWATER AT ORBA
HEALTH CENTRE
IN UDENU LOCAL GOVERNMENT AREA OF ENUGU
STATE

APRIL, 2017




Joseph O. A.
20/4/17
Geologist

GEOPHYSICAL AND HYDROGEOLOGICAL INVESTIGATION FOR GROUNDWATER AT ORBA HEALTH CENTRE IN UDENU LOCAL GOVERNMENT AREA OF ENUGU STATE

1.0 INTRODUCTION

- 1.1 The ensuing geophysical investigation report is the result of field Vertical Electrical Sounding (VES) carried out in April, 2017 for the sinking of a successful borehole at Orba Health Centre in Udeni Local Government Area of Enugu State.
- 1.2 Pre-drilling geophysical surveys happens to be the scientific method of determining the existence or otherwise of underground water. This therefore, necessitated the commissioning of consulting hydrogeologists to carry out that survey. The vertical electrical sounding was therefore carried out at the proposed site on the 19th April, 2017.
- 1.3 This report involves studies of the physiographic, local geology and hydrogeology as well as the conduct of vertical electrical sounding of the subsurface vertical stratigraphy.

2.0 LOCATION

Orba Health Centre is located in Udeni Local Government Area. Globally, it lies on Longitude 007°28.125 East of the Greenwich Meridian and on Latitude 06°51.568 North of the Equator.